

NAUTILUS PBM FIELD GUIDE

Your Roadmap to **Fiduciary-Aligned**
Pharmacy Benefits



Contract Protections



Cost Controls



Clinical Quality



Foreword

Your Roadmap to Fiduciary-Aligned Pharmacy Benefits · 2026 Edition

Pharmacy benefits are broken. This Field Guide helps you fix them.

This is a first-of-its-kind open-source initiative with templates, tools, and tactics to take back control of drug spending, strengthen compliance, and deliver better outcomes for plan participants. Whether you're renegotiating your PBM contract, testing the market, or running a full RFP, it's your roadmap for getting the decisions right.

The Three Things You Need, In One Place

This edition connects three Nautilus resources that work together.

- Use the **PBM Field Guide** for understanding the procurement and contracting process.
- Read **The Middleman's Cut** for the industry context and business case for change.
- Run **Contract X-Ray** to see where your own contract stands against fiduciary standards.

Resource	What It Gives You
PBM Field Guide	The operating manual. The framework, the procurement steps, the contract language, and the oversight discipline that turn intent into a defensible process.
The Middleman's Cut	The narrative and the evidence. How the PBM model came to work against the people it was built to serve, told through named cases, real filings, and the numbers behind them.
Contract X-Ray	The scored answer. An analysis service that reads your actual PBM contract against fiduciary-aligned standards and shows you, provision by provision, where it protects participants and where it falls short.

About Nautilus

Nautilus Health Institute is an independent 501c(3) nonprofit advancing fiduciary-aligned standards and tools that help employers evaluate, negotiate, and improve PBM contracts. We take no money from PBMs. We sell no vendor placements. Our mission is to put the standards and technology that were once reserved for the largest and best-advised plans into the hands of every fiduciary.

Employer-first. Independent expertise. No vendor conflicts.

Learn more and reach us at www.nautilushealth.org/PBM

Like this Field Guide? Share it. Use it. Help fix what's broken.

Contents

Executive Summary	1
Pharmacy Benefits Are Bleeding Money and Exposing You to Liability	1
Introduction	3
Your Complete Playbook	3
Inside You'll Find.....	3
What Makes This Different	3
Part 1: Why This Matters	4
The Stakes Are Personal.....	4
Part 2: Understanding PBMs	7
How PBMs Make Money.....	7
Part 3: Building a Fiduciary Foundation	11
The Duty: Protect Participants and the Plan.....	11
Part 4: Clinical Stewardship in Action.....	13
Managing Costs and Conflicts	13
Part 5: Overcoming Barriers to Change	17
The 95/5 Reality: Why Change Feels Risky.....	17
Part 6: The Fiduciary Procurement Roadmap	21
Procurement Is Where Theory Meets Action.....	21
Part 7: Data Access and Audit Rights	26
Data Access Is Now Federal Law.....	26
Part 8: Contracts That Protect	29
Why Contracts Matter More Than Promises	29
What's Next: From Insight to Action	33
The Stakes Are Rising. Your Path Is Clear.	33
Where to Start.....	33
The Field Guide Toolkit	34
Lead the Change.....	34
Panel of Expert Contributors	35

Executive Summary

Pharmacy Benefits Are Bleeding Money and Exposing You to Liability

The bottom line: Pharmacy benefits now represent roughly a quarter of health spend for the average employer and your largest unmanaged fiduciary risk. Individual executives are being named in federal class-action lawsuits over pharmacy oversight. Large employers are forecasting health-cost increases driven almost entirely by pharmaceuticals. The cost of inaction has never been higher.

The accelerating crisis: Pharmacy's share of health spending climbed from roughly 21% to 27% between 2021 and 2023. Nearly all recent health-cost growth now comes from pharmacy.

The prize: Employers who take control typically achieve 15% to 30% cost reductions while removing personal legal exposure. Independent PBM audits regularly recover \$2 to \$8 per member per month in contract violations and pricing errors, often reaching back several years.

The Hidden Crisis

The three largest PBMs control about 80% of the market. The top six control 95%. That structure is built to extract profit from plan sponsors. A small slice of the market offers genuine transparency and better outcomes at lower cost, if you know how to find and evaluate it.

The traditional incumbents built an empire on three hidden revenue streams:

- **Spread pricing.** Charging you more than they pay pharmacies.
- **Rebate retention.** Keeping manufacturer payments meant for your plan.
- **Conflicted advisors.** Paying your consultants to recommend their services.

PBMs are the granddaddy of healthcare pricing shenanigans.

– Dave Chase, Founder of Health Rosetta

Don't let those shenanigans make their way into your plan.

The legal risk is real and growing. Under ERISA, executives already face personal liability for PBM oversight failures, and the CAA 2026 reforms raise the bar further. Recent lawsuits name individual fiduciaries for failing to audit conflicts, monitor performance, or confirm transparent pricing. If your advisors can't certify in writing they accept zero PBM compensation, you're making decisions on conflicted advice, and you may be out of step with federal law.

The Immediate Opportunity

Financial returns: 15% to 30% cost reductions through proper procurement, \$2 to \$8 per member per month in audit recoveries, and measurable gains in vendor performance.

Risk protection: a personal-liability shield through documented fiduciary process, compliance with federal transparency requirements, and a competitive edge from superior benefit design.

The market shift: roughly one-third of employers will reassess their PBM in the coming year. That's the largest pharmacy-procurement disruption in decades.

Why Most Employers Stay Trapped

Three barriers lock employers into bad arrangements. Conflicted advice, because some advisors profit from the status quo. Information asymmetry, because PBMs control the data you need for oversight. And contract complexity, because the legal language is written to obscure rather than clarify. This Guide dismantles all three.

Your Next Step

You don't need a year to begin. You need two moves.

1. **Clean house on advice.** Within 30 days, request written compensation disclosure from every advisor and consultant. Keep the ones who fully disclose vendor payments. Replace the ones who won't.
2. **Get your baseline.** Run your current PBM contract through Contract X-Ray at contractxray.com. In under an hour you'll see how it scores against fiduciary-aligned standards, and you'll know which provisions need attention before you do anything else.

Two moves, thirty days, no new budget. That's where the turnaround starts.

Introduction

Your Complete Playbook

This Guide is for:

- **C-suite executives and board members** protecting against personal liability.
- **Benefits and finance leaders** pursuing 15% to 30% cost reductions.
- **Advisors and consultants** ready to lead the change.
- **Any fiduciary** preparing for a PBM contract decision.

Inside You'll Find

- **The Six Pillars framework**, mapped to the ten provisions and three domains that Contract X-Ray scores. Strategy and measurement in one system.
- **Model contract language** and redline-ready clauses that create real accountability.
- **A procurement roadmap** that produces a defensible fiduciary record from first RFI to signed contract.
- **Contract X-Ray**, the analysis service that scores your actual contract, and the **Nautilus PBM Configurator**, the interactive tool that runs your vendor RFI and RFP evaluation.

What Makes This Different

This is an operational toolkit built by practitioners: PBM experts and ERISA attorneys who've helped employers achieve documented results. It doesn't stop at theory. It hands you the framework, the scoring, and the language to act.

Part 1: Why This Matters

The Stakes Are Personal

Fiduciary duty isn't abstract. If you sponsor a health plan, it's your name on the line. Employees have sued Johnson & Johnson and Wells Fargo, alleging their benefits fiduciaries failed to control what the plan paid its PBM. Courts dismissed the early complaints on standing grounds, but the theory is spreading, with a similar suit filed against JPMorgan in 2025. These are personal liabilities, and they can follow you long after you leave the company.

The Consolidated Appropriations Act of 2026 and existing ERISA standards make one thing clear: plan sponsors must act in the sole interest of participants and beneficiaries. When you can't say how your PBM makes money, or whether your advisor is conflicted, you're falling short of that standard.

Critical Facts Every Fiduciary Must Know

- **You don't need the title to carry liability.** Discretion over plan assets is fiduciary duty.
- **ERISA claims often fall outside D&O coverage.** The legal exposure is personal.
- **ERISA judgments follow you.** They can't be discharged in bankruptcy.
- **Ignorance is no defense.** Courts expect a documented oversight process.
- **You're held to expert standards.** Whether you have the expertise or not.

Compliance Is a Strategic Advantage, Not Just a Legal Shield

Employers who adopt fiduciary standards for PBM management routinely see lower plan costs through smarter procurement, better access and adherence for employees, stronger clinical outcomes for high-risk populations, and reduced legal and reputational risk for leadership. The investment generates real savings while it protects you legally. That combination is rare in the compliance world.

The Law Already Points One Direction

Most employer health plans are governed by ERISA, the same law that governs retirement plans. It imposes strict fiduciary obligations on anyone with discretion over plan assets or decisions.

Federal law arrived in two waves. The Consolidated Appropriations Act of 2021 put three operative duties in front of every plan sponsor. All three are in force today.

In Force Today: What CAA 2021 Requires	Why It Matters
Compensation disclosure. Advisors and service providers must disclose direct and indirect compensation.	PBM-linked payments to your advisors must be documented. Undisclosed conflicts are a compliance failure, not a gray area.
Data access. Contracts may not block your access to price, quality, and claims information.	Gag clauses that once hid pricing and rebates are prohibited. Blocking your data is now a violation.
Prescription-drug reporting. Plans must submit pharmacy pricing and rebate information to federal agencies.	You stay accountable even when the PBM handles the filing. Delegation doesn't transfer the duty.

The CAA 2026 reforms, signed February 3, 2026, build on that baseline. They are the most consequential PBM legislation in a generation, and the core provisions apply to contracts entered into or renewed for plan years beginning on or after August 3, 2028. For calendar-year plans, that means January 1, 2029.

What CAA 2026 Adds	Why It Matters
PBM compensation disclosure. PBMs become ERISA covered service providers and must disclose all direct and indirect compensation.	The disclosure duty that already covers your advisors extends to the PBM itself. Hidden or unreasonable compensation becomes a prohibited transaction.
Full rebate pass-through. PBMs, including their rebate aggregators and group purchasing organizations, must remit 100% of rebates, fees, and other remuneration to the plan. Transparent, quantifiable fees for bona fide services are the only exception.	Full pass-through stops being a negotiating win and becomes the statutory default, with audit rights to confirm the money actually arrives.
Semiannual plan reporting. Detailed reports on drug spend, rebates, spread pricing, and steering toward PBM-affiliated pharmacies. Quarterly on request.	The data you need for oversight arrives on a federal schedule, backed by civil penalties for late or false reporting.

CAA 2026 doesn't replace ERISA. It activates ERISA for pharmacy benefits. These are legal requirements, not suggestions, and they apply whether you run the plan yourself or delegate to a TPA. Regulators and plaintiff attorneys are watching, and courts have been plain: you can't blindly rely on vendors. The 2028 dates are closer than they look. A standard three-year PBM contract signed this year will still be in force when the new requirements land, so negotiate the disclosures, the pass-through, and the audit rights in now.

Knowing your obligations is half the work. The other half is harder, because the PBM ecosystem is designed to make compliance difficult.

The Deception Problem

The traditional PBM market runs on opacity built to extract profit at your expense: spread pricing hidden from plan sponsors, rebates that reward kickbacks over care, and contracts that block audits and data access. The result is a system of misaligned incentives and unchecked conflicts.

The Advisor Problem

Conflicted brokers and consultants often make the problem worse. Brokers are frequently paid by carriers or PBMs rather than by the employer. Some consultants require PBM payments to be included in an RFP. Others embed \$1 to \$5 per-script fees in the admin structure. Many purchasing coalitions are run by brokers who profit from the deals they steer. These arrangements are rarely disclosed, which runs against CAA 2026 transparency requirements. If an advisor can't or won't explain how they're paid, they shouldn't be advising your plan.

The Stacked Deck

Most employers didn't knowingly sign up for this. The problem isn't the benefits team. The system is built to exploit complexity and hide conflicts. Traditional PBMs profit from confusion. Conflicted advisors have a financial reason to defend the status quo. And the vendors who could solve these problems often never get invited to compete.

This Field Guide changes the equation. It points toward a new model: one that arms fiduciaries with real tools, removes dependence on conflicted middlemen, and rewards transparency, stewardship, and measurable impact.

BOARDROOM TAKEAWAY

This Risk Is Real, and Fixable

The bottom line: Pharmacy benefit oversight has moved from best practice to legal requirement, with personal liability for executives who fail to act. The upside: the same disciplined oversight that shields you legally is what drives the savings.

Action required: Treat PBM oversight with the same rigor you apply to 401(k) management. Document your process, require transparency, and hold vendors accountable with enforceable contract terms. Start by scoring your current contract so you know where you stand.

You can't fix what you can't see. Part 2 pulls apart how PBMs make money, where market concentration breeds conflict, and why even a self-described transparent alternative may not be what it seems.

Part 2: Understanding PBMs

How PBMs Make Money

PBMs were built to process pharmacy claims. Today they're middlemen who extract profit all along the supply chain. The traditional model draws revenue from sources that create built-in conflicts with your plan's goals of affordability, access, and transparency.

The fiduciary trap: if you can't explain how your PBM is paid, how that shapes its decisions, and whether the compensation is fair and reasonable, you may be falling short of your duties under ERISA and CAA 2026.

The more confusing the model, the easier it is to extract profit, and the harder it becomes to meet your fiduciary duties.

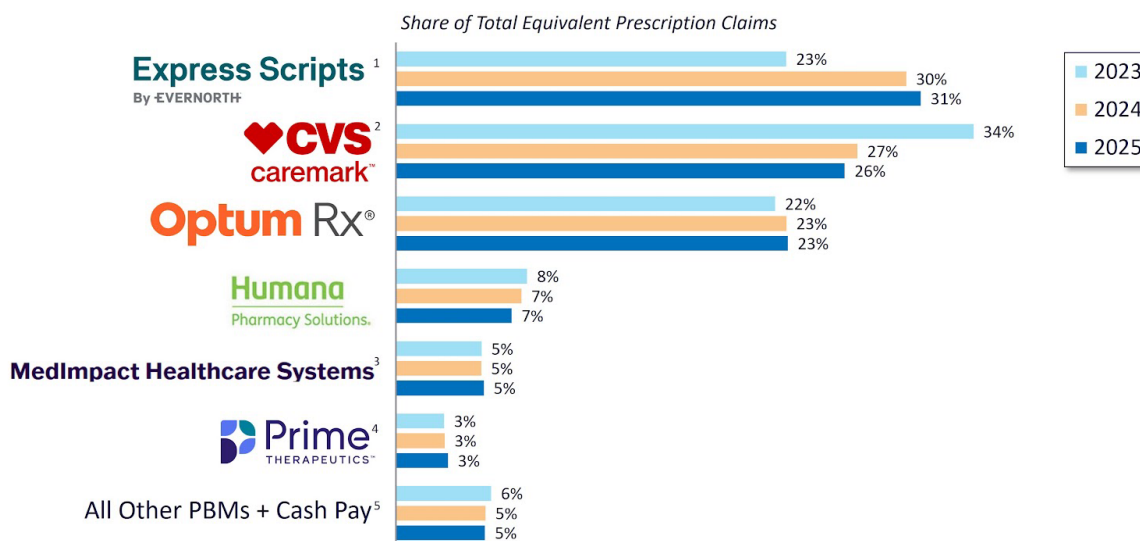
Revenue Source	What It Looks Like	Fiduciary Risk
Rebate retention	PBM keeps part of manufacturer rebates	Inflated drug costs, high costs for sickest employees
Spread pricing	Charges the plan more than it pays pharmacies	Hidden margins, opaque financials
Formulary distortion	High-rebate drugs favored over lower-cost options	Clinical harm, misaligned incentives
Hidden fees	Baked into claims, reporting, or network charges	Masked compensation, audit obstacles
Pharmacy steering	Mandatory use of PBM-owned mail or specialty	Inflated cost, reduced local access
Data gag clauses	Limits on pricing, rebate, or contract access	Blocks oversight and benchmarking

Market Concentration and Built-In Conflicts

Three companies control nearly 80% of the PBM market, each embedded in a large healthcare conglomerate: CVS Caremark inside CVS Health, Express Scripts inside Cigna, and OptumRx inside UnitedHealth Group.

The next tier, including Humana Pharmacy Solutions, MedImpact, and Prime Therapeutics, rounds out a top six that together controls about 95% of the market.

PBM Market Share, By Total Equivalent Prescription Claims Managed, 2023 to 2025



Source: Drug Channel Institute March 2026

Vertical Integration Builds the Conflict In

The largest PBMs sit inside conglomerates that also own insurers, pharmacies, and provider groups. That structure creates conflicts by design: rebates captured by affiliated companies rather than returned to your plan, patients steered to owned mail-order and specialty pharmacies, formularies optimized for internal margin over clinical value, and your claims data monetized to feed other business lines.

Vertical Business Relationships Within the U.S. Drug Channel, 2026

	BlueCross BlueShield	THE CIGNA GROUP	CENTENE Corporation	CVS Health	Élevance Health	Humana	UNITEDHEALTH GROUP
Insurer ASO/TPA platform ¹	BlueCross BlueShield luminare health	cigna healthcare Allegiance ²	Medicaid wellcare ambetter	aetna Meritain Health ^{an aetna company}	Anthem Wellpoint AmeriBen	Humana	United Healthcare UMR
PBM	Prime ³ THERAPEUTICS	Express Scripts By EVERNORTH	CENTENE PHARMACY SERVICES ⁸	CVS Caremark	carelon ⁹ Rx	Humana Pharmacy Solutions	Optum Rx [®]
GPO	synergie ⁴	Ascent ^{Health Services} Econdisc ^{Contractual Services}	—	zinc RED OAK SOURCING	synergie ⁴	—	EMISAR
Manufacturer	—	Qualient ^{Pharmaceuticals}	—	cordavis	—	—	nuvaira
Wholesale distribution	—	CuraScript SD By EVERNORTH	—	—	—	—	Optum Frontier Therapies
Specialty / Mail pharmacy	Prime Therapeutics Pharmacy ⁵	Accredo By EVERNORTH Express Scripts Pharmacy Freedom Fertility By EVERNORTH CAREPATH ⁶	AcariaHealth ^{Specialty Pharmacy}	CVS specialty Mail order pharmacy services	carelon ⁹ Rx BioPlus ^{A Carelon Company}	CenterWell Specialty Pharmacy CenterWell Pharmacy	Optum Specialty Pharmacy Optum Frontier Therapies cps ¹⁰
Retail/LTC pharmacy	—	—	—	CVS pharmacy Omnicare ¹¹ a CVS Health company	—	—	genOa healthcare PHARMSCRIPT
Provider	—	Evernorth Behavioral Care Group MD Live By EVERNORTH Village ⁷	Community Medical Group Magellan HEALTH	CVS minute clinic signifyhealth Oak St. Health	carelon ⁹ Health carelon ⁹ Behavioral Health	CenterWell Senior Primary Care CenterWell Home Health conviva ^{Senior Primary Care}	Optum

Source: Drug Channel Institute April 2026

Group Purchasing: Scale Can Hide the Same Problems

In pursuit of scale, many employers join coalitions, group purchasing organizations, or buying groups. The promise is savings. The reality often replicates the same power dynamics and opacity found in the big three. Watch for broker control, where the coalition is run by advisors who profit from hidden fees. Watch for undisclosed compensation layered on top. And watch for group savings that mask individual overpayment.

The Advisor Problem: Conflicts and Capture

Many advisors tasked with helping you evaluate PBMs are conflicted themselves: override commissions from carriers, pay-to-play requirements where a PBM must pay to be considered, partnership arrangements that bundle PBM services, one-to-five-dollar per-script fees embedded in admin structures, and a failure to disclose any of it in writing. Fiduciary-aligned procurement starts with unconflicted advice. If an advisor can't answer the compensation question clearly, find one who can.

The Challengers: Independent and Transparent PBMs

Roughly 5% to 10% of the market is served by independent PBMs that avoid the vertically integrated model and compete on transparency. These vendors typically accept a flat administrative fee as their only compensation, offer pass-through pricing with no spread or rebate retention, return 100% of rebates to the plan, provide real-time claims feeds and full audit rights, and own no retail pharmacies, insurers, or specialty channels. Buyer beware: transparency is easy to claim in a pitch deck and harder to prove in a contract. If they say they're transparent, they should be ready to prove it. Running your contract through contractxray.com can reveal these conflicts.

The Supporting Cast: Unbundled Solutions

You no longer need one PBM for every function. Employers are disaggregating services and building specialized solutions: independent clinical review free from dispensing conflicts, specialty drug management focused on high-cost oversight, pharmacy navigation that helps members find the lowest-cost option, independent contract and claims analysis with no vendor ties, rebate optimization firms that recover and validate manufacturer payments, and alternative sourcing through cost-plus models like Mark Cuban Cost Plus Drugs and nonprofit consortiums like Civica Rx. One caution: many of these can be white-labeled through the big three, repackaging the same conflicts. Verify true independence and the compensation model.

CONTRACT X-RAY

Independent scoring, not another sales pitch. Contract X-Ray reads your actual PBM contract against fiduciary-aligned standards and returns a provision-by-provision. It scores explicit contract language only, not marketing claims or verbal promises.

The three-file transparency test starts the conversation. Contract X-Ray finishes it. Ask your PBM for the three files that separate real transparency from marketing: pharmacy claims, rebate data, and administrative fees. Then let the contract itself tell you whether the transparency is enforceable.

Send your contract to contractxray.com. Get the score in under an hour with Quick Look, or the complete fiduciary picture with a Full Assessment. Delivered as a service. No software, no platform, no learning curve.

BOARDROOM TAKEAWAY

The Market Is Concentrated, But Alternatives Exist

The bottom line: Understanding PBM business models is fiduciary homework, not academic curiosity. Three companies control 80% of the market through vertical integration that hides conflicts and inflates cost. A transparent minority offers a real alternative, if you can tell the difference.

Action required: Score the contract through contractxray.com to confirm the transparency is written down and enforceable, not just promised.

Part 3: Building a Fiduciary Foundation

The Duty: Protect Participants and the Plan

Fiduciary-aligned procurement starts with one duty: act solely in the interest of plan participants and beneficiaries. Under ERISA, that means aligning your drug benefit strategy with health outcomes, cost stewardship, and legal compliance, with the care, skill, prudence, and diligence of a responsible expert.

Three facts every fiduciary carries into the room:

1. **You can't delegate accountability.** Advisors don't bear the fiduciary duty. You do. Courts don't ask who picked the vendor. They ask what you did to provide oversight.
2. **You can be held personally liable.** Executives, board members, and HR leaders have been named for failing to oversee PBMs, monitor contracts, and enforce fiduciary protections.
3. **Process is what's judged.** Courts evaluate whether you followed a prudent process, not whether you picked a theoretically perfect vendor.

The Six Pillars of Fiduciary-Aligned PBMs

The Six Pillars are your strategic framework for evaluating any PBM arrangement. They protect participants, reduce cost, and create a defensible decision process.

Pillar	What It Requires
1. Clinical Stewardship	Formulary decisions grounded in clinical evidence and patient outcomes. Effectiveness, cost, and access over rebate optimization. Independent review of high-cost therapies.
2. Full Financial Transparency	Every dollar of compensation disclosed. No spread pricing, rebate retention, or hidden fees. Contractual rights to audit all financial arrangements.
3. Unconflicted Procurement	Advisors and vendors free from conflicts. Compensation from the employer, not PBMs or carriers. No pay-to-play fees or steering incentives.
4. Data Ownership and Protection	Plan sponsors own and control their data for oversight and benchmarking. Real-time access to claims, pricing, and rebate data. No limits on portability.
5. Local Access and Provider Fairness	Networks that support patient access and fair provider pay. No steering to owned facilities without clinical justification. Sustainable reimbursement for all pharmacy types.
6. Attestation and Oversight	Vendors certify compliance and submit to independent verification. Annual attestation, audit rights with extrapolation, and performance guarantees with financial penalties.

These pillars are not just aspirational. Each can be reviewed and scored by Contract X-ray. A panel of benefits and PBM experts have developed a scoring rubric to score and compare vendor contracts to model contract language. Each provision scores from 0 to 100. The overall score is an evenly weighted average of the ten. Scores map to five readiness bands, from Red Flag through Concern, Fair, Good, and Excellent. Excellent is rare and meaningful.

Across the Nautilus reference set, the practical ceiling sits in the low 90s, and only a handful of scored contracts reach 90 or above. When a report puts your contract in a band, you're seeing where it lands against real market contracts, not against a theoretical ideal.

CONTRACT X-RAY

The Fiduciary Alignment Score (FAS) is the headline 0-to-100 number on your Contract Scorecard, the evenly weighted composite of all ten provisions. It's the metric you benchmark, negotiate against, and improve over time.

The Data Sovereignty Score reports how your contract performs on the five provisions that govern data control, and the designation it earns: Gold, Silver, Bronze, or Ineligible. This is what turns the Six Pillars from a philosophy into a scorecard.

From Pillars to Priorities: Build Your Evaluation Framework

The Six Pillars define your strategy. Your priorities decide how you weigh each element. Start your fiduciary defense file by documenting those priorities. Which pillar matters most to your plan's current risks and member needs? What evidence would prove a vendor truly delivers on it? How will you measure and document performance over time?

Those decisions become the foundation for everything downstream: which vendor questions matter most, how you weight topics in scoring, how you justify the selection, which contract terms are non-negotiable, and how your oversight meetings tie back to the framework.

If Financial Transparency is your top priority, you'll weight pricing questions heavily, treat the three-file test as a gate, make audit rights non-negotiable, and document every vendor answer about compensation. Your pillar priorities aren't just strategy. They're the documented record that defends every decision that follows.

BOARDROOM TAKEAWAY

Know What Good Looks Like, Then Prove You Applied It

The bottom line: The Six Pillars define fiduciary excellence, and the ten-provision scoring model makes it measurable. Your documented priorities and evaluation process are what defend the decision if it's ever questioned.

Action required: Use the Six Pillars to set your priorities and the Contract X-Ray provisions to score against them. Document your rationale and build evaluation criteria that reflect your plan's specific needs. This becomes both your RFP foundation and your legal defense.

Part 4: Clinical Stewardship in Action

Managing Costs and Conflicts

Clinical Stewardship, the first pillar, demands active oversight of the financial and operational tactics that distort clinical decisions. Here's how to put it into practice.

Start With the Biggest Distortion: Break Rebate Addiction

Rebates sound like savings. In practice they often inflate list prices, favor high-rebate drugs over clinically superior generics, drive formulary decisions on margin rather than evidence, and push cost and conflict onto the sickest patients. A rebate-driven formulary is a liability waiting to happen.

The traditional model rewards opacity, inflated list prices, and convoluted contracts while patient outcomes slip. The fiduciary alternative is value-based design grounded in clinical evidence. Focus on lowest net cost after all rebates and discounts, not the highest gross rebate.

Fix Generics First: The Foundation of Affordable Care

Generics are the biggest opportunity for immediate impact. They're effective, essential, and inexpensive, and they should carry minimal or no member cost-sharing. They must never be excluded to protect a high-rebate brand.

- **Benefit design:** zero-dollar copays for maintenance generics like hypertension, cholesterol, and diabetes therapies, plus pricing tools that steer members to the lowest-cost option.
- **Transparent pricing:** require ingredient-cost transparency (NADAC plus a reasonable dispensing fee), eliminate spread pricing on generics, and ban MAC-list manipulation.
- **Alternative sourcing:** consider direct-pay cash pricing for high-deductible members and independent pharmacies that price generics transparently.
- **Formulary protection:** prevent exclusion of generics in favor of high-rebate brands and watch for improper reclassification as specialty generics.

If your members pay more than \$5 for common generics, your PBM is failing them.

Specialty Drugs: The Fastest-Growing Risk Category

Specialty drugs account for more than half of total pharmacy spend in many plans and grow 10% to 15% a year. Not all use is appropriate, and not all cost is justified. The challenge is complexity that requires specialized handling and support, cost volatility where single claims commonly exceed \$100,000 a year, access barriers from prior authorization and site restrictions, and outcomes uncertainty where real-world results may not match the trials.

Fiduciary oversight framework:

- **Independent clinical review:** remove PBM conflicts from prior-authorization decisions, require licensed clinicians, demand medical records rather than physician attestations alone, and verify diagnosis, prior therapies, and comorbidities before approval.
- **Site-of-care optimization:** hospital outpatient departments often charge two to five times more than alternatives, home infusion frequently costs less with a better patient experience, and white-bagging or brown-bagging can remove facility buy-and-bill markups.

Biosimilars: Order in the Market Chaos

The biosimilar market is expanding fast and creating opportunity and risk daily. When six or seven biosimilars launch at once for a single drug, the lowest price can shift weekly while PBMs and brand manufacturers fight to protect high-margin status quo. Some PBMs now own biosimilar manufacturing, which creates new steering incentives, and brand makers deploy enhanced rebate deals designed to block biosimilar uptake.

A biosimilar-first strategy has four parts:

- **Real-time monitoring:** track launches and net cost after rebates, not just WAC, and flag PBM-owned biosimilars that may create steering conflicts.
- **Formulary agility:** build quarterly review for high-volume biologics, prefer biosimilars unless clinical evidence says otherwise, and require justification for excluding a lower-cost biosimilar.
- **Clinical safeguards:** protect patient stability through switching protocols and monitor continuation rates and real-world effectiveness.
- **Contract protection:** negotiate shared-savings arrangements, prevent undisclosed PBM retention of biosimilar-driven savings, require advance notice of formulary changes, and put conversion rates into performance guarantees.

GLP-1s: A Fiduciary Stress Test in Real Time

Ozempic, Wegovy, Mounjaro, and Zepbound dominate headlines and health-plan spending. The clinical promise is real, and so are the financial and fiduciary risks. This is a real-time stress test of your oversight.

The class delivers proven value for type 2 diabetes and obesity, with a pipeline reaching into sleep apnea, addiction, heart failure, and kidney disease, and the potential for real medical-cost offsets. It also carries tradeoffs: results vary widely, some patients discontinue within months, real-world cost per sustained responder can exceed projections, and supply constraints complicate care and formulary design. Inappropriate use adds risk through off-label cosmetic prescribing, thin medical oversight, and missing lifestyle integration.

- **Clinical oversight:** require comprehensive prior authorization with medical records, verify appropriate diagnosis and failure of alternatives, and monitor for contraindications.
- **Financial management:** model in lowest net cost rather than inflated rebate projections, account for discontinuation rates, and track start rates, adherence, and early discontinuation in real time.

Your PBM's cost projections may assume higher success rates than the real-world data supports.

Formulary Oversight: Take Back Clinical Control

Traditional PBM formularies aren't clinical documents. They're profit strategies, often designed to maximize rebates, inflate margin, or drive traffic to affiliated pharmacies. Watch the P&T committee for non-clinical PBM employees or rebate-tied consultants, no employer input or review rights, minutes and voting records kept off-limits, unclear override pathways, and decisions that prioritize rebate maximization over safety, effectiveness, and cost.

Evaluate on three fronts:

- **Clinical appropriateness:** evidence-based selection over rebate optimization, independent review of therapeutic alternatives, and integration with your population's needs.
- **Tiering transparency:** understand how drugs are classified, watch for generics reclassified as specialty to inflate cost, and demand clinical rationale for tier placement.
- **Exclusions and step therapy:** confirm override pathways don't require legal intervention and that step therapy serves clinical appropriateness, not cost deflection.

Independent Pharmacies: Access, Economics, and Duty

Independent pharmacies deliver essential care beyond dispensing, especially in rural and underserved areas: counseling, chronic-care support, vaccinations, and services that improve adherence. Today's PBM system undercuts them through unfair reimbursement and opaque pricing. Spread-pricing games reimburse independents below cost while charging plans above market, and vertical integration lets PBMs pay their own pharmacies more while steering patients to owned mail-order and specialty channels.

A fair network strategy uses NADAC plus a dispensing fee, eliminates spread pricing and MAC manipulation, and requires sustainable reimbursement for all pharmacy types. It protects access by maintaining adequate independent representation, prohibiting steering to owned facilities without clinical justification, and supporting member choice.

Clinical Program Evaluation: Stewardship or Slideware

Every PBM promotes clinical programs. The question is whether they improve outcomes or inflate margin. Too often they're lightly customized, opaquely priced, and more profitable for the PBM than valuable for the plan.

- **Evaluate the evidence base:** baseline data on the problem and its cost, clear and measurable objectives, and ROI reporting for your plan rather than a generic case study.
- **Evaluate the economics:** is pricing per participant, per outreach, or per outcome, and are there financial penalties when a program underdelivers.
- **Evaluate the integration:** does the program share data with your TPA and care management, and can members opt out without penalty.

Managing Pharmacy Cost Volatility

Pharmacy is now the most volatile line item in many employer health plans. A single specialty or gene-therapy claim can reach into the hundreds of thousands of dollars, and new launches can trigger budget shocks no traditional forecast anticipated. That volatility is both a fiduciary risk and an oversight opportunity.

The pipeline reality. In 2025, the FDA approved 46 novel drugs, and oncology and rare-disease therapies now account for more than 85% of the development pipeline. Specialty drugs are projected to exceed 60% of total pharmacy spending in 2026 while reaching fewer than 5% of members. Cost escalation is accelerating: the average specialty drug now runs \$200,000 to \$400,000 a year, recent approvals land between \$240,000 and \$480,000 depending on patient weight, and the newest gene therapies reach \$2 to 4 million per course.

Volatility comes from three sources. Specialty pipeline dominance, as gene and cell therapies enter faster and indication expansions like GLP-1s create new demand waves. Medical-versus-pharmacy coordination gaps, as high-cost drugs run through the medical benefit with fewer

controls and slower cost visibility. And stop-loss misalignment, as many stop-loss contracts weren't designed for large single pharmacy claims.

Management strategies:

- **Monitor predictively:** track the FDA approval pipeline, now dominated by specialty products, watch member demographics for high-risk conditions, and build early-warning systems for trend changes.
- **Coordinate financial protection:** confirm your stop-loss covers prescription drugs, review it for specialty exclusions and medical-benefit gaps, negotiate trend caps or risk corridors for volatile categories, and build contingency reserves.
- **Close the medical-pharmacy gap:** apply enhanced prior authorization on both sides, coordinate utilization management, consider white-bagging or brown-bagging to shift medical drugs under pharmacy oversight, and use independent clinical review to verify medical necessity regardless of benefit classification.

Advanced Cost Management: Real Value, Real Guardrails

Rising specialty cost is driving new programs. International importation, manufacturer assistance, and alternative funding can deliver substantial savings, and each carries risks that require review and oversight.

International importation can save 30% to 70% on high-cost medications when handled correctly. It fits stable maintenance therapies sourced from Tier 1 countries such as Canada, the UK, and Australia, where the price differential justifies the operational complexity. Success requires regulatory compliance, supply-chain security, and clinical safety protocols. Handled carelessly, it creates safety and legal exposure that outweighs the savings.

Alternative funding through patient assistance programs and charitable foundations can cut specialty cost when structured properly. The fiduciary challenge is telling legitimate cost-saving programs apart from formulary-manipulation schemes that disrupt members or create compliance risk. Evaluate legal compliance with commercial insurance terms, member impact, and vendor compensation. Avoid programs that intentionally make drugs uncovered, vendors paid on claims diverted rather than outcomes achieved, or any arrangement without a clear legal framework.

BOARDROOM TAKEAWAY

Clinical Stewardship Is Your Fiduciary Duty

The bottom line: Formularies are care pathways that shape member health and plan cost. With specialty at roughly three-quarters of the pipeline and individual claims reaching well into six and seven figures, passive clinical oversight exposes you to heavy financial and legal risk.

Action required: Break rebate addiction first: require lowest net cost over highest rebate. Put independent clinical review on specialty. Confirm your stop-loss covers prescription drugs. Advanced strategies exist, and each needs rigorous fiduciary evaluation before you adopt it.

Part 5: Overcoming Barriers to Change

The 95/5 Reality: Why Change Feels Risky

The psychological challenge is real. You're considering leaving 95% of the market to choose from the other 5%. That feels risky until you see it clearly: the 95% is built to extract profit from your sickest employees, and the 5% is built to serve them.

This is human impact, not abstract risk.

It's your sickest employees that pay for those rebates. When they can't afford their deductible, they're sicker. God help us if somebody in your company suffers the same fate as that 22-year-old kid in Wisconsin who died because he couldn't afford his inhaler. You think your CEO is going to want to be on the front page of the Wall Street Journal because somebody died because he wanted a bigger rebate?

– Mark Cuban, Mark Cuban Cost Plus Drugs

De-Risk the Transition

1. **Start with disclosure. Clean house first.** Secure unconflicted advice before any vendor decision. Require full compensation disclosure from advisors and consultants, and change the relationship with anyone who won't certify they accept no PBM compensation.
2. **Set your baseline with data.** Score your current contract with Contract X-Ray to establish a fiduciary baseline, then build a current performance baseline from claims for accurate comparison and set clear targets for the next contract.
3. **Remediate before you replace.** Test the incumbent first. See whether they'll accept fiduciary-aligned terms. A refusal becomes part of your business case for change.
4. **Test the market.** Issue a Market Test RFI to fiduciary-aligned alternatives, apply the three-file transparency test to separate marketing from reality, and build a short list from PBMs with CAA 2026 Ready contracts.
5. **Learn from peers who've done it.** Connect with employers who made similar transitions and use their lessons to set targets and avoid pitfalls.
6. **Require cost and performance guarantees.** Seek PMPM guarantees, financial penalties for missed savings, and remedies including early termination if performance disappoints.
7. **Run a rigorous process.** Follow the procurement process in Part 6 and build a complete audit trail that demonstrates prudence.

Common Barriers, and How to Clear Them

Barrier	The Reality	The Move
Fear of disruption	Your current formulary already prioritizes rebates over outcomes. That's the real disruption.	Require disruption analysis by NDC and GPI before any transition, grandfather existing therapies, and build member transition support into selection criteria.

Barrier	The Reality	The Move
The loss-of-scale myth	The largest PBMs often deliver the worst net value.	Compare lowest net cost after all fees and rebates, not gross discounts. Consider consortium arrangements with aligned employers.
The expertise gap	Current complexity is intentional, designed to obscure vendor profit.	Engage independent advisors with fiduciary-aligned experience and join peer groups making similar moves.
Advisor resistance	Advisors work for whoever pays them, and it should be you.	Require full compensation disclosure, a fiduciary pledge from any advisor influencing selection, and consider fee-only advisors.
Executive hesitation	The boat is already taking on water. You're patching the hull.	Frame the change as fiduciary compliance, present benchmarking data on overpayment, and name the legal exposure of passive oversight.
Bundle comfort	Bundling creates blind spots: no audit rights, no formulary control, no rebate visibility.	Carve-out gives you direct contracts, more control, and integrated claims feeds. Independent PBMs coordinate medical and pharmacy routinely.

Manage the CEO Conversation

When you propose moving from the 95% to the 5%, your CEO will press you. Consider giving them a copy of [*The Middleman's Cut*](#) to prep the conversation. Then answer with confidence.

Frame the change as fiduciary compliance, not vendor dissatisfaction. That's the difference between a defensible decision and a preference.

The CEO asks	You answer
Why take the risk of changing what's working?	It's working for our PBM, not us. Those rebate checks are funded by our employees rationing insulin and skipping doses. We're not taking on risk. We're reducing it.
Won't we lose leverage with a smaller PBM?	The three largest PBMs deliver the worst net value because they profit from complexity. Net cost after all fees matters more than gross discounts. Many companies save 15% to 30% by moving to transparent pricing. That's real leverage.
What if this disrupts member care?	We'll run member-level disruption analysis before any change and grandfather existing therapies. The real disruption is a formulary that prioritizes rebates over outcomes.
How do we know the savings are real?	We'll require performance guarantees that our total cost won't exceed current spend. If they don't save us money, they work for free.
What's our legal exposure?	The greater exposure comes from not acting. CAA 2026 makes PBM oversight legally required, and courts are holding executives personally liable for passive oversight.

Recognize Vendor Sales Tactics During Procurement

Once an RFP starts, traditional PBMs deploy predictable tactics to protect the status quo. Name them and counter them.

The tactic	Your counter
Proprietary-information claims that block transparency requests	Make specific data-delivery capabilities a contract requirement, with penalties for delay.
System-limitation excuses that data can't be produced in usable form	Require the three-file transparency test (pharmacy claims, rebate data, admin fees) as a qualification gate. No exceptions.
Additional-cost barriers for data access beyond standard services	Include all data access and audit cooperation in base contract pricing.
Industry-standard deflection on restrictive terms	Point to fiduciary-aligned alternatives that accept transparent terms.
Legal-review delays that slow negotiation	Set specific timelines with escalating penalties.
Special-relationship appeals to skip competitive evaluation	Apply the same evaluation criteria regardless of incumbent status.

Red flags that signal misalignment: inability to provide transparency files within 48 hours, contract terms that can't be modified under competitive pressure, guarantees that offer credits instead of financial penalties, and pricing that takes multiple meetings to explain. If they make procurement difficult, they'll make oversight impossible.

Build Market Momentum for Change

Individual moves matter, and market transformation takes collective action. Leading employers build momentum through the National Alliance of Healthcare Purchaser Coalitions, which is active on PBM issues, and through peer collaboration: shared evaluation criteria and benchmarks, coordinated RFP timing for leverage, exchanged lessons, and joint education for advisors and vendors.

On the vendor side, require attestation of fiduciary alignment, share performance data so other employers can decide well, decline to work with advisors who hide conflicts, and back vendors who prove genuine transparency.

CONTRACT X-RAY

The market has a scoreboard. The CAA 2026 Readiness Report, sponsored by the National Alliance of Healthcare Purchaser Coalitions, reports how PBM contracts measure against the fiduciary obligations of CAA 2026. It's built on Contract X-Ray and the Fiduciary Alignment Score, published quarterly, and it grows more authoritative every issue as the scored population expands.

It's not a ranking or an endorsement. It's a market picture: how ready the contracts in the market actually are. Your participation helps set the standard.

BOARDROOM TAKEAWAY

The Market Is Changing. Lead or Follow.

The bottom line: The biggest barrier to change isn't market availability. It's internal inertia and conflicted advice. The transparent minority delivers better outcomes at lower cost, once you can counter vendor tactics and internal resistance.

Action required: Use the seven-step de-risking sequence to turn a leap of faith into a documented business decision. Start with advisor disclosure, set your baseline with a contract score, test the incumbent on fiduciary terms, then evaluate alternatives through the process in Part 6.

Part 6: The Fiduciary Procurement Roadmap

Procurement Is Where Theory Meets Action

PBM procurement isn't a solo act. It's documented, collaborative decision-making that demonstrates a prudent process. Courts don't just judge the outcome of your selection. They scrutinize whether you assembled the right expertise, followed a systematic approach, and maintained oversight throughout. The prudent-expert standard holds you to the decisions a knowledgeable professional would make, whether or not you have that expertise on staff. Committee composition and process documentation become central to your legal defense.

Two Tools, Two Jobs

This roadmap uses two Nautilus tools that do different work. Keep them straight and each earns its place.

Tool	The Job It Does
Contract X-Ray	Scores the actual contract. Point it at your incumbent contract and at any finalist's proposed contract to see, provision by provision, where the language protects participants and where it falls short. This is the objective read on the paper you'll actually sign. Available at contractxray.com
Nautilus PBM Configurator	Runs the vendor evaluation. Builds your RFI and RFP from an expert-curated question library, weights topics to your priorities, organizes vendor responses, and generates scorecards. This is how you evaluate what vendors say and promise during the process. Available at nautilushealth.org/pbm

CONTRACT X-RAY

Score the contract, then run the process. Start every engagement by scoring the incumbent contract with Contract X-Ray. That single step gives you a fiduciary baseline, a prioritized list of weak provisions, and the evidence to open a serious conversation. Then use the Configurator to structure the RFI, RFP, and scoring that surround the decision.

At the finish line, score leading vendors proposed contract before you make a decision. The vendor's pitch and the vendor's contract are not the same document. Contract X-Ray reads the one that binds you.

Build Your Evaluation Committee

Successful procurement needs cross-functional expertise and pharmacy-benefits knowledge.

Role	Key Responsibilities	Critical Phases
Fiduciary Committee	Fiduciary oversight, major-decision approval, risk governance	Key milestones, final approval
Executive Sponsor	Process oversight, decision authority	Selection, contract approval

Role	Key Responsibilities	Critical Phases
HR / Benefits	Project management, employee impact	All phases
CFO / Finance	Cost analysis, ROI validation, budgeting	Evaluation, selection
Legal Counsel	Contracts, compliance, risk assessment	Contract negotiation
IT / Data	Data requirements, system integration	Technical evaluation

Advisory requirements. Most organizations lack sufficient internal PBM expertise, so independent advisory support is essential. Pharmacy-benefits expertise is mandatory, whether through experienced internal staff or independent consultants with demonstrated PBM knowledge. Independent, fee-only consultants usually give the most objective guidance. And every advisor must disclose compensation sources in writing.

Tools of the Trade

Good procurement combines the right expertise with the right tools: question libraries that cover every critical area, response templates vendors can complete, scoring frameworks that reflect your priorities, documentation systems that build an audit trail, scorecards for review and presentation, and comparison tools for side-by-side analysis. You can build these yourself, work with specialized consultants, or use purpose-built tooling from Nautilus

NAUTILUS PBM CONFIGURATOR

The Nautilus PBM Configurator packages all of this. It builds customized, fiduciary-aligned RFI and RFP documents in minutes and can cut procurement time and effort in half while it improves decision quality. Request access at www.nautilushealth.org.

The Two-Track RFI Strategy

Fiduciary procurement often opens with an RFI, and not every RFI serves the same purpose.

Track 1: Contract Compliance RFI, for your incumbent. Ask your current PBM to identify any terms that fall short of fiduciary standards, propose revised terms to close the gaps, and certify their compensation, conflicts, and auditability. Pair this with a Contract X-Ray score of the incumbent contract so the gaps are documented in scored, provision-level detail. When a vendor refuses fiduciary-aligned terms, that refusal becomes your documented justification for seeking alternatives.

The Contract Compliance RFI template provides ten targeted questions developed by PBM experts and ERISA attorneys for exactly this assessment.

Track 2: Market Test RFI, for alternatives. At the same time, issue a Market Test RFI to alternative PBMs to gauge capabilities, service models, and compliance alignment, narrow the list of vendors invited to the RFP, and set market benchmarks against your incumbent. You're not committing to switch. You're exercising your duty of prudence.

The Market Test RFI template provides 40 questions across 11 topics, with a question library of 285+ questions prepared by a panel of PBM experts to draw from.

Build a Defensible Scoring Framework

Fiduciaries must show a prudent process. That means documenting the topics evaluated, the weight assigned to each, and the scoring method used. The scoring framework is your legal defense: courts examine whether you followed a systematic, documented evaluation that put participant interests first.

Essential elements: topic coverage across clinical, financial, operational, and compliance factors; weighted priorities based on your plan's needs; standardized scoring on a consistent 1-to-5 scale with clear criteria; and complete documentation of decisions and rationale. Build enough questions to cover every critical area without overwhelming evaluators or vendors, and keep scoring consistent across reviewers with a clear audit trail.

The Configurator lets you assign weights to topics and multipliers to individual questions so your methodology reflects your priorities. Its question dashboard shows how well selected questions cover each topic area and whether you have the right mix of yes/no and open-ended questions for a balanced evaluation.

Prepare and Manage Vendor Responses

Systematic preparation drives accuracy and efficiency. Assemble questions into a structured response format vendors can complete, provide clear instructions on format and deadlines, distribute identical templates to every vendor, and set a Q&A protocol for clarifications. On the way back, track submission status, organize responses by topic for efficient review and calibration, flag incomplete submissions, and structure everything for scoring. Preparation often determines evaluation quality: well-structured templates produce accurate responses and faster review.

The Configurator organizes questions into Excel-based vendor response templates and manages submissions by topic, removing the manual formatting and organization work.

Smart Scorecards: Turn Opinions Into Decisions

Once responses are organized by topic, systematic scoring turns subjective judgment into objective comparison. Score yes/no questions as 0 for a wrong answer and 5 for a right one. Score descriptive questions 0 to 5 on the quality of the answer. Multiply each score by its question multiplier, then compare raw against weighted scores to see the tradeoffs. Roll question scores into topic scores, apply topic weights, and roll those into totals. Generate both summary scorecards, with ranked totals and topic scores for executive review, and detail scorecards, with question-level scores, vendor responses, and reviewer comments. Together they let stakeholders see the big picture, examine the detail, and defend the rationale if it's ever questioned.

The Configurator generates summary and detail scorecards and automates the calculations. Heat maps highlight scoring differences, and pinning enables side-by-side response comparison for final calibration.

Red Flags During Vendor Evaluation

Spotting vendor problems during evaluation is core fiduciary diligence. Document them carefully, because they become either grounds for disqualification or priority items for negotiation.

Transparency red flags: inability to provide the three-file transparency test, evasive or incomplete answers, reluctance to provide attestations, and complex explanations you can't independently verify.

Process red flags: advisor pressure to exclude vendors without valid reason, vendors who bypass your process to lobby executives, last-minute special offers meant to shortcut evaluation, and references who can't produce specific performance data.

Contract red flags: standard agreements with no willingness to negotiate, vague definitions subject to unilateral change, audit or data-access limitations, and guarantees with no financial teeth.

The Configurator's vendor dashboard lets reviewers examine every response and leave detailed comments, building a documented trail of concerns that supports scoring decisions and negotiation.

Force Apples-to-Apples Comparisons

The biggest procurement trap is letting PBMs control their own math. Each uses different assumptions, definitions, and baselines, which makes fair comparison impossible. You have three ways to regain control.

1. **Price using the three files. Most accurate, and preferred.** Request complete historical claims, rebate, and fee data, analyze the PBM's demonstrated pricing patterns, apply that performance to your population, and validate marketing promises against actual results.
2. **Reprice it yourself. Preferred when the three files aren't available.** Request unit prices, discount terms, rebate guarantees, and fees, then keep full control of assumptions and formulas and apply identical assumptions to every vendor.
3. **Standardized repricing by the PBMs. Least preferred.** Provide identical raw claims to every bidder, require repricing using your definitions, normalize the results, and accept the risk that comes with vendor-generated math.

The more control you keep over assumptions and calculations, the more reliable the comparison. Options one and two give you the most accurate results because you control the analysis.

Validate, Interview, and Shortlist

Scoring is the start, not the finish. Shortlist on substance and hold incumbents to the same standard. Validate marketing claims through references and performance guarantees. Interview to pressure-test the team behind the proposal: clarify vague responses, assess day-to-day fit, and surface hidden risk before it becomes a regret.

Document Your Decision

After scoring and interviews, reconvene the committee. Finalize scores with documented rationale, connect each to the fiduciary pillars with an explicit link to participant-first decision-making, and record who was involved, how the process worked, and why the choice was made. Your executive summary should include total weighted scores with topic breakdowns, a lowest-net-cost comparison rather than gross pricing, the key advantages and disadvantages of the top candidates, and the risk-mitigation plan for the chosen vendor.

Align Scorecard and Contract

Don't lose leverage at the finish line. Your scorecard is also your negotiation roadmap. High-weighted topics become contract priorities. Flag non-negotiables such as no spread pricing and 100% rebate pass-through. Eliminate vendors that fail binary deal-breakers. Focus negotiation on the areas where vendors scored poorly. You can't score vendors on transparency and then

accept an opaque contract, or score them on audit rights and then sign away access. Before you sign, run the proposed contract through Contract X-Ray so the scorecard priorities and the contract language line up in scored detail. Model contract language for core provisions can speed the negotiation while protecting what you evaluated for.

BOARDROOM TAKEAWAY

Process Is Your Protection

The bottom line: Your procurement process is your fiduciary defense. A systematic, documented approach delivers better benefits at lower cost while it turns vendor selection into a defensible business decision. The right tools can cut execution time in half, and proper expertise is non-negotiable for decisions that withstand scrutiny.

Action required: Assemble a committee with pharmacy expertise, control your pricing comparisons, and document every decision with clear rationale. Score both the incumbent and the winning contract with Contract X-Ray, and let your scorecard priorities drive the negotiation. The process you follow matters more than the vendor you pick.

Part 7: Data Access and Audit Rights

Data Access Is Now Federal Law

Federal law already gives employers both a legal right and a fiduciary duty to access the data behind their health plan: pharmacy claims, rebates, contract terms, and performance guarantees. ERISA and the 2021 transparency rules put that duty in force today, and CAA 2026 strengthens it from 2028 with mandatory PBM reporting. This is federal law, backed by personal liability for fiduciaries who fail to comply.

What Data You Must Demand

Fiduciary oversight requires data that's complete, timely, and usable. At minimum, you need access to four categories.

- **Claims and utilization.** All pharmacy claims at NDC-level detail, medical claims for physician-administered drugs (J-codes), prior-authorization requests, approvals and denials, and appeal and grievance records.
- **Pricing and financial.** Ingredient cost and dispensing-fee breakdowns, MAC lists and pricing-methodology documentation, plan payments and member cost-sharing, and all rebates, fees, and manufacturer payments by drug.
- **Network and provider.** Pharmacy participation agreements and reimbursement rates, network adequacy and access metrics, and provider performance and quality measures.
- **Clinical and formulary.** The complete formulary with tiers and restrictions, P&T minutes and decision rationale, utilization-management criteria and approval rates, and clinical-program participation and outcomes.

Don't accept summary reports when you need source data. Aggregated data hides the problems you're responsible for finding.

Audit Rights: Your Most Underused Weapon

Audit rights are the enforcement mechanism that turns contract promises into verifiable performance. Without them, you're relying on vendor honesty, and that isn't a fiduciary strategy. Many employers hold some audit rights and never use them, missing both cost recovery and vendor accountability.

Common audit failures that protect vendors:

- Limited-scope restrictions and reasonable-effort clauses that let the vendor decide what you can examine.
- Proprietary-information exclusions that block key pricing data.
- No extrapolation rights, so recovery is capped at the sample.
- PBM-approved-auditor requirements that limit independence.
- Weak remediation that offers credits instead of cash.

What fiduciary-grade audit rights include:

- Unlimited scope and frequency, with access to all relevant records, including downstream vendor agreements.
- Independent auditor selection without vendor approval, including PBM audit firms.

- Extrapolation and recovery rights: authority to project sample findings, automatic reimbursement for confirmed errors within set timeframes, and interest on delayed refunds at commercial rates.

CONTRACT X-RAY

Your Data Sovereignty Score puts a number on this pillar. Contract X-Ray scores the five provisions that govern data control and audit, then reports the designation your contract earns: Gold, Silver, Bronze, or Ineligible. Two gates must both clear. Every one of the five provisions has to meet its floor, and the overall Fiduciary Alignment Score has to clear the threshold. Neither compensates for the other.

It's the fastest way to know whether the data rights in Part 7 are actually written into your contract, or only assumed.

The Data Oversight ROI

Data access and audit rights aren't only compliance. They generate savings. Immediate recovery runs \$2 to \$8 per member per month from identified overpayments and violations, with 15% to 25% overpayment common through spread pricing and hidden fees, and documented audit savings that often exceed cost by 10 to 20 times.

The ongoing benefits compound: a 15% to 30% reduction in unexpected cost spikes through early trend identification, a 20% to 40% improvement in vendor service levels once vendors know they're monitored, a stronger negotiating position backed by performance data, and a defensible oversight record. Few compliance investments pay for themselves while strengthening legal protection. This one does.

From Rights to Results

Rights mean nothing without a system to use them.

- **Monthly:** automated exception reporting and trend analysis, vendor performance measured against guarantees, cost and utilization benchmarking, and member-impact monitoring.
- **Quarterly:** total-cost-of-ownership calculation and competitive benchmarking, clinical-program ROI validation, formulary net-cost review, and contract-compliance assessment.
- **Annually:** a full vendor performance review with a renewal decision analysis, market-alternatives evaluation, fiduciary-compliance documentation and legal review, and strategic planning for benefit design and vendor management.

Build Your Legal Defense File Through Data

Data access is also how you build your defense. Courts evaluate fiduciary performance on the process you followed and the information you had. Keep the records that prove active oversight: monthly data-review minutes with attendees and decisions, vendor scorecards with trend analysis and corrective actions, cost and utilization reports with peer comparisons and variance explanations, audit results and remediation tracking with timelines, and annual fiduciary-committee reports with recommendations and approvals. Document your use of the data, and you document your oversight.

BOARDROOM TAKEAWAY

You Can't Manage What You Can't See

The bottom line: Data access and audit rights are the foundation of oversight and profit recovery. Federal law makes access legally required, with CAA 2026 adding mandatory reporting from 2028, and vendor compliance isn't automatic. Most employers hold some audit rights and never use them, leaving \$2 to \$8 per member per month in recoverable overpayment on the table.

Action required: Demand complete, real-time access to all four data categories, conduct independent audits with extrapolation rights, and document everything for legal defense. Confirm the rights are scored into your contract with a Data Sovereignty Score. Your most underused weapon is also your most profitable.

Part 8: Contracts That Protect

Why Contracts Matter More Than Promises

Fiduciary oversight doesn't stop at vendor selection. It lives in every line of your contract. A great RFP doesn't guarantee a great outcome. Only a great contract does. PBM contracts are legally binding documents that determine how your pharmacy dollars are spent, who controls your data and clinical decisions, whether your plan meets ERISA and CAA 2026 requirements, and what recourse you have when a vendor underperforms.

The strongest oversight tool isn't a quarterly meeting or an annual report. It's the contract. A well-structured PBM contract codifies fiduciary protections and performance expectations, prevents hidden fees and surprise terms from surfacing later, secures the data access and audit rights that make oversight real, provides remedies for noncompliance, and creates accountability through enforceable financial consequences. Weak contracts are a root cause of overspending, poor performance, and fiduciary exposure.

Common Contract Failures That Expose Plan Sponsors

Most PBM contracts are written to protect the vendor. Knowing the failure patterns helps you avoid the traps that create liability.

Contract Failure	What It Looks Like	Fiduciary Risk
Disguised gag clauses	Limits on sharing pricing, rebate, or performance data despite the CAA 2026 ban	Illegal under federal law, blocks benchmarking
Audit restrictions	PBM approval of auditors, scope limits, or access fees	Prevents verification of compliance
Ambiguous pricing	Terms like customary pricing or undefined MAC methods	Enables spread pricing and cost manipulation
Rebate loopholes	Vague definitions that let the PBM keep administrative fees	Hidden revenue inflates cost
Data-ownership gaps	Unclear rights to claims data or portability limits	Blocks oversight and benchmarking
Weak performance terms	No guarantees, or penalties with no financial teeth	No recourse when service fails
Bundled vendor roles	Same entity as PBM, TPA, and stop-loss carrier	Conflicts with no accountability

Key Elements of a Fiduciary-Grade Contract

Element	Why It Matters	What to Watch For
Scope of services	Defines what's in the base price	Vague terms like customary services that enable upcharges
Pricing model	Determines transparency and total cost	Hidden spread or undefined discounts

Element	Why It Matters	What to Watch For
Rebate transparency	Full pass-through of manufacturer payments	Aggregation, admin offsets, delayed reporting
Audit rights	Independent financial and operational oversight	PBM approval, scope limits, or fees
Data ownership	Who owns and can access the data	Ambiguous ownership or system restrictions
Performance guarantees	Ties fees to service and outcomes	Credits instead of financial penalties
Termination rights	Protects your exit and transition	Long notice, punitive fees, thin transition help
Network flexibility	Access to high-value and local pharmacies	Mandatory steering or restrictive networks
Clinical governance	Formulary serves participants, not rebates	PBM control of P&T with no employer input
Compensation disclosure	Required under ERISA and CAA 2026	Incomplete disclosure of direct and indirect revenue

Performance Guarantees: Beyond Marketing Theater

Most PBM guarantees are built to sound protective while delivering little accountability. Real protection requires financial consequences. Four principles: financial enforcement over service credits, measurable baselines and targets, escalating penalties for repeated failures, and independent verification through audit rights. Effective guarantees cover four areas: cost and financial performance, clinical and access performance, service-level performance, and transparency and compliance performance.

The Strategic Redline Approach

Contract negotiation is where procurement leverage becomes legal protection.

Phase 1: Establish non-negotiables. Before you send redlines, document the terms fiduciary compliance requires: 100% rebate pass-through with comprehensive definitions, unrestricted audit rights with independent auditor selection, real-time data access and ownership, and no spread pricing or hidden fees. Present these as compliance requirements, not negotiating preferences.

Phase 2: Propose model language. Don't just name problems. Bring solutions. Start from proven model contract language, require the vendor to accept, reject, or counter each clause in writing, document the rationale for anything they won't accept, and escalate noncompliance to senior vendor management.

Phase 3: Document responses. Build a clear paper trail: track which terms the vendor accepts without change, record objections and proposed alternatives, separate acceptable compromise from deal-breakers, and keep a decision log showing how each issue resolved.

CONTRACT X-RAY

The Negotiation Report hands you the redlines. As part of a Full Assessment, Contract X-Ray produces a Negotiation Report: a gap analysis with model contract language and prioritized talking points, tied to the specific provisions where your contract scores low. It turns a score into the exact language to put in front of the vendor.

Use it as the backbone of your Phase 2 redline. The provisions that scored lowest are the terms that need the language most.

Implementation and Transition Protections

Transition planning is fiduciary risk management. Members who can't access medications during a vendor change create immediate legal exposure and real medical emergencies. Your contract must treat implementation as seriously as pricing, with specific protections, timelines, and financial consequences for failures.

- **Go-live:** require a parallel-processing period with the incumbent, set success criteria for claims, member access, and networks, build in rollback provisions, and assign responsibility for processing errors during transition.
- **Disruption analysis:** require member-level disruption analysis with grandfathering, advance notice of formulary or network changes, member-communication protocols with timelines, and override mechanisms for members facing access disruption.
- **Data migration:** require complete, validated transfer of all plan data, reconciliation with accuracy thresholds, backup retention by the outgoing vendor, and defined liability for loss or corruption.
- **Runout and support:** require extended runout services at no added cost, dual-vendor coordination for a smooth member experience, emergency support mechanisms, and transfer of historical claims and rebates to the successor.

Ongoing Review and Re-Procurement

Signing is where the real work begins. Fiduciary oversight requires continuous monitoring of performance and compliance. Review performance against guarantees and service levels on a set cadence, re-check compensation disclosures for new revenue sources, evaluate market conditions and alternative capabilities, and document performance and compliance issues as they arise.

Keep the right to modify terms for regulatory change, a process to add or remove services without full renegotiation, escalation procedures for disputes, and a mechanism to implement audit findings.

Trigger a new RFP when performance consistently misses guarantees, when the market offers materially better alternatives, or when ownership or relationship changes create new conflicts.

BOARDROOM TAKEAWAY

Your Contract Is Your Most Powerful Tool

The bottom line: Great procurement means nothing without a great contract. Weak contracts unravel even a perfect selection, because vendor promises aren't binding and contract terms are. The contract decides whether you get transparency or opacity, accountability or excuses, savings or hidden fees.

Action required: Embed fiduciary protections in every provision, require financial penalties for failures, and treat implementation as seriously as pricing. Score the final contract before signing, and use the Negotiation Report to redline from the provisions that score lowest. If it isn't prohibited in writing, assume it's allowed.

The roadmap is complete: the risks, the framework, the procurement process, the data rights, the contract protections. *What's Next turns all of it into a first move you can make today.*

What's Next: From Insight to Action

You now hold what most employers lack: a practical roadmap to transform broken pharmacy benefits while strengthening compliance and reducing cost. You've seen how the system became distorted, and why smart employers are rebuilding it from the inside out, cutting cost while improving member outcomes and legal protection.

You've worked the whole framework: spot the hidden risks in traditional PBM models and conflicted advice, build a defensible procurement process, put data access and audit rights to work, negotiate contracts that protect your plan, and tell a truly fiduciary-aligned vendor from one that just markets like one. Knowledge isn't action, though. Now it's time to lead.

The Stakes Are Rising. Your Path Is Clear.

Every month you wait, your plan loses money to spread pricing and retained rebates, your liability grows as scrutiny intensifies, your members pay more while vendors profit from opacity, and other employers gain an edge by moving first. The tools to fix this are ready, and the return is immediate. Your first move is small and specific: score your current contract, and clean house on advisor conflicts. Everything else follows from knowing where you stand.

Where to Start

Three entry points. Read the story, run the scores, and see how the market measures up.

Resource	What It Is and Where to Find It
Contract X-Ray	Score your PBM contract against fiduciary-aligned standards. Quick Look returns a free dashboard in under an hour. Full Assessment delivers the complete picture: Contract Scorecard, Fiduciary Alignment Score, Negotiation Report, Executive Brief, and Data Sovereignty Score. Start at contractxray.com .
The Middleman's Cut	The companion book. The narrative and the evidence behind everything in this Guide. Builds the impetus for change with executives
CAA 2026 Readiness Report	The market-level publication, sponsored by the National Alliance of Healthcare Purchaser Coalitions. How PBM contracts measure against CAA 2026. Quarterly. Learn more at contractxray.com/caa-2026 .
Nautilus PBM Configurator	The interactive tool that builds your fiduciary-aligned RFI and RFP and runs vendor scoring. Request access at nautilushealth.org/pbm

The Field Guide Toolkit

Visit www.nautilushealth.org/PBM for more information and other Nautilus resources like these:

Nautilus Resource	What It Gives You
Model Contract Language	Redline-ready clauses for the most important fiduciary, operational, and financial provisions
Evaluation Question Bank	Full library of PBM evaluation questions, 285+ across 11 topics, prepared by a panel of PBM experts. The raw material for your RFI and RFP.
Data Specification Request	Ready-to-send template that specifies the exact data files and fields to demand from any PBM.
PBM Company List and Profiles	Directory of PBMs with profiles covering ownership, pricing model, and transparency posture, so you know who competes for your business.
Case Studies: Employers Who Took Control	Real transitions from employers who moved to fiduciary-aligned arrangements, and the results they achieved.
Executive FAQ	Plain-language answers to the questions your CEO, CFO, and board will ask, ready to drop into a briefing.
The Biggest Unmanaged Risks in Rx Benefits	Short briefing on the exposures most plans miss, from spread pricing to specialty volatility.
Mark Cuban's Advice to CEOs	Cuban's blunt guidance to employers, drawn from his public remarks. A fast way to make the stakes concrete.
Curated Industry Content	A reading list of the reporting and research behind this Guide.

Guidance and Contact

For role-specific starting points, see contractxray.com and select plan-sponsors, advisors, or PBMs. Learn more about the methodology at contractxray.com/methodology. To talk it through, reach us at info@nautilushealth.org.

Lead the Change

Most employers are flying blind, relying on conflicted advisors and accepting what their PBM tells them. You now have the knowledge and the tools to lead differently. Fiduciary excellence is doing the right things deliberately, transparently, and in your participants' sole interest.

The next move is yours, and your participants are counting on it.

Panel of Expert Contributors

This Field Guide exists because of the brainpower and boldness of its contributors. If you shared your time, insight, or critique, thank you. This is your work as much as ours.

Nautilus Health Institute PBM Project is led by Steve Ditto with an advisory panel that includes experts across the industry.

Contributors: Alan Pannier, Andrew McNerlin, Anthony Pudlo, Bryce Platt, Catalina Gorla, David Mastrangelo, Dustin Conrad, Erik Davis, Jake Frenz, Jen Foley, Joey Dizenhouse, John Rogers, Jon Schlosser, Levi Roseman, Lisa Krajewski, Logan Lyle, Mark Mincy, Michael Navin, Mike Stancil, Paul Holmes, Pramod John, Rachel Strauss, Randy Vogenberg, Ryan Rice, Sam Slaughter, Scott Haas, Sean Morrison, Tim Thomas, Timon Wang, Traci Poole, Wes Hartig, and Zack Robinson.

The CAA 2026 Readiness Report is produced in partnership with the National Alliance of Healthcare Purchaser Coalitions.

Just discovering the movement? The door's still open. Join us to help shape the future of fiduciary-aligned PBMs at nautilushealth.org/PBM